

Wisconsin Department of Health Services

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: 0020912	(X2) MULTIPLE CONSTRUCTION A. BUILDING: _____ B. WING: _____	(X3) DATE SURVEY COMPLETED C 07/29/2026
NAME OF PROVIDER OR SUPPLIER WESTON MEMORY CARE		STREET ADDRESS, CITY, STATE, ZIP CODE 3404 COMMUNITY CENTER DRIVE WESTON, WI 54476		
(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETE DATE
N 000	Initial Comments On 07/23/2026, Surveyor conducted 2 complaint investigations at Weston Memory Care. Data was collected through 07/29/2026. One of the 2 complaints was substantiated. Four deficiencies were identified. Census: 57	N 000		
N 362	83.33(1)(d) Grievance procedure: written summary The CBRF shall provide a written summary of the grievance, the findings and the conclusions and any action taken to the resident or the resident ' s legal representative and the resident ' s case manager. The CBRF shall maintain a copy of the investigation. This Rule is not met as evidenced by: Based on record review and interview, the provider did not provide a written summary of a grievance with the findings, conclusion, and any action taken to Resident 1's legal representative. Findings include: On 05/27/2026, the Department received a complaint with allegations that a resident was observed to have multiple bruises on his/her body without an explanation. On 07/23/2026, Surveyor reviewed Resident 1's record.	N 362		

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

TITLE

(X6) DATE

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N 362	Continued From page 1 Resident 1 had multiple diagnoses including dementia with behavioral disturbance. S/he had an activated health care power of attorney, Person B. On 05/06/2026 at 4:15 a.m., staff documented: "Writer noticed while giving resident [his/her] morning medication that [s/he] had a large redness on [his/her] lower chin and the same in between [his/her] eyes on the bridge of [his/her] nose. Unsure what exactly the redness is/where it came from, but it appears and feels like a possible skin rash/rug burn. On-call sent a message and incident report filed." On 05/11/2026, staff documented: "[Resident 1] was discharged from the system. Discharge Reason: Deceased Notes: Deceased - Notes: Resident passed on 5/10/2026." On 07/23/2026, Surveyor reviewed an incident investigation for an injury of unknown source, which was dated and signed by Administrator A on 07/23/2026. The summary read: "Summary Based on the investigation, were there any contributing factors? Resident consistently attempted to self-transfer and ambulate. [S/he] would also reposition [him/herself] often when sleeping throughout the night. Describe action taken/measure initiated to reduce the possibility of future incidents: Resident has been placed on 30-minute checks to ensure [s/he] is positioned safely and to reduce the risk of any future fall." On 07/23/2026 at approximately 2:45 p.m., Surveyor interviewed Administrator A. Surveyor	N 362		

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N 362	Continued From page 2 asked when s/he started the investigation into Resident 1's injuries. Administrator A said s/he started the investigation on 05/11/2026. Administrator A said Resident 1's family wanted the injury investigated to ensure there was no misconduct. Surveyor requested documentation to show when Administrator A started the investigation as the report given to Surveyor was dated 07/23/2026. On 07/27/2026, Administrator A emailed Surveyor with additional documentation including a "Grievance/Investigation Form". On 07/27/2026, Surveyor reviewed the grievance investigation. The investigation included a "Resident/Family Statement", which was dated 05/11/2026. The statement read: "Family requested an investigation to be completed due to misconduct that they suspect happened to their [parent]." This form indicated the family statement was from Person B, who was Resident 1's legal representative. The form was signed by Administrator A as the person recording the statement. The conclusion, dated 05/18/2026, and signed by Administrator A read: "After investigatios [sic] with staff and review of [Resident 1's] case notes it is suspected that this is a self inflicted [sic] wound. [Resident 1] would self propel through out [sic] the facility and would bump into walls and was very restless." On 07/27/2026 at approximately 3:15 p.m., Surveyor interviewed Person B on the phone. Person B told Surveyor s/he went into Resident 1's room and noticed the marks on Resident 1's face. Person B said s/he went to Administrator A and asked what happened. Person B said s/he was told, they were not sure what happened. Person B said Resident 1 was no longer	N 362		

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N 362	Continued From page 3 ambulatory after s/he was hospitalized in March 2026. Person B said s/he and other family members looked over Resident 1's body. Person B told Surveyor they had found other bruises and marks on Resident 1's body. Person B said s/he was mostly concerned about the bruising between Resident 1's eyes. Person B said s/he did not know if Resident 1 was hit and had a head injury and if that may have contributed to Resident 1 dying shortly after the bruises were identified. Person B told Surveyor s/he was emailing Administrator A and asking about the investigation. Person B told Surveyor that Administrator A did not respond to his/her emails. Person B said s/he did speak to Administrator A on the phone, this last week. Person B said s/he did not receive a written summary of the investigation. On 07/29/2026 from approximately 8:30 a.m. to 9:30 a.m., Surveyor interviewed Administrator A on the phone. Administrator A told Surveyor s/he communicated the conclusion of the investigation into caregiver misconduct on the phone with Person B. Administrator A said s/he did not provide a written summary of the grievance to Person B.	N 362		
N 388	83.35(3)(c) Implement, follow the individual service plan Implementation. The CBRF shall implement and follow the individual service plan as written. This Rule is not met as evidenced by: Based on record review and interview, the provider did not ensure Resident 1's individual service plan (ISP) was implemented as written.	N 388		

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N 388	Continued From page 4 Resident 1's ISP indicated s/he had a bed alarm. Resident 1's bed alarm did not function properly on multiple occasions. Findings include: On 05/27/2026, the Department received a complaint with allegations that a resident was not receiving adequate care and services. On 07/23/2026 through 07/29/2026, Surveyor reviewed Resident 1's record. Resident 1's face sheet indicated s/he was admitted to the facility on 02/03/2026. S/he had multiple diagnoses including: brain aneurysm, cerebral vascular accident (stroke), mixed anxiety depressive disorder, and dementia with behavioral disturbance. Resident 1 had an activated health care power of attorney. Resident 1's ISP, titled "Other: Bed alarm," dated 03/13/2026, signed by Resident Health Coordinator (RHC) D, read, in part: "Resident utilizes a bed alarm. Resident needs this to alert staff when resident is attempting to transfer without assistance and to minimize the risk of serious injuries related to falls. Resident's Desired Goals & Outcomes: Resident desires to remain free from serious injuries or incidents from occurring. Service Provider Responsibilities: Staff" (Staff was the only word after Service Provider Responsibilities). Resident 1's medication administration record (MAR) included an order for alarm checks. The order read, "ALARM CHECKS Check placement of bed alarm to ensure proper positioning of pad, placement should be under buttock. Ensure alarm is on and functioning properly. If not, check	N 388		

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N 388	Continued From page 5 batteries and/or alert RHC (resident health coordinator)/DON (director of nursing)." This order included the checks to be done at 7:00 a.m., 3:00 p.m., and 11:00 p.m. On 03/25/2026 at 6:23 a.m., staff documented in the MAR notations for alarm checks: "Works but rings very frequently without resident moving." On 03/29/2026 at 4:45 p.m., staff documented in the MAR notations for alarm checks: "Check observation note regarding this." On 03/29/2026 at 6:15 p.m., staff documented an observation note, which read: "Resident's bed alarm does work but it does not at the same time. Writer noticed when resident stands up and gets off bed alarm it does not go off, but when resident sits back down onto bed alarm it will go off." On 04/02/2026 at 9:45 a.m., staff documented in the MAR notations for alarm checks: "Alarm is still not working properly. Management has been informed." On 04/12/2026, an incident report indicated Resident 1 had an unwitnessed fall at 2:21 p.m. The report read: "Resident was yelling, staff went in and found [him/her] on the floor next to [his/her] bed." The report did not indicate if the alarm was sounding. On 04/12/2026 at 3:56 p.m., staff documented in the MAR notations for alarm checks: "Resident's alarm box for alarm is missing. No where in room." On 04/12/2026 at 11:11 p.m., staff documented in the MAR notations for alarm checks: "Resident does not have bed alarm due to alarm	N 388		

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N 388	Continued From page 6 malfunction." From 04/13/2026 until 04/25/2026, staff documented their initials on the MAR indicating they performed an alarm check, except for on 04/21/2026, the 3:00 p.m. check was blank. On 04/26/2026 at 11:02 p.m., staff documented in the MAR notations for alarm checks: RHC alerted to malfunctioning bed alarm. Resident is on 30-minute checks." On 04/27/2026, staff documented their initials on the MAR indicating they performed an alarm check at 7:00 a.m., 3:00 p.m., and 11:00 p.m. On 04/28/2026, staff documented their initials on the MAR indicating they performed an alarm check at 7:00 a.m. and 3:00 p.m. On 04/29/2026 at 12:38 a.m., staff documented in the MAR notations for alarm checks: "Unable to locate bed alarm." On 04/29/2026 at 7:00 a.m., staff documented their initials on the MAR indicating they performed an alarm check." On 04/29/2026, an incident report indicated Resident 1 had an unwitnessed fall at 2:31 a.m. the report read, in part: "As writer and staff were entering hallway, writer and staff heard resident yelling for help. Resident was wedged between their bed side table and their bed sitting in an upright position with their body faced towards the wall and their back towards their room. Resident was sitting on their legs and feet. Writer and staff asked what happened, resident was unable to provide clear response. After performing range of motion, resident was transferred to their bed and	N 388		

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N 388	Continued From page 7 their vitals were collected. No visible marks or scratches present, doesn't has [sic] no visible indication of hitting their head but ultimately unclear response when asked. When transferring, resident began to dry heave and had an [sic] very small amount of emesis. Resident winced, clenched teeth, and said 'ow' a few times but once vitals were gathered and they were covered in bed, they appeared far less uncomfortable. On-Call contacted as well as [family] ... Previous shift stated supervisors were aware of lack of bed alarm for resident. When contacted, supervisor informed writer they had not been made aware and instructed writer to monitor more frequently, that they'd look at things in the morning." On 07/27/2026, Surveyor emailed Administrator A and asked questions related to Resident 1, including the following: "6. On 03/29/26 and 03/31/26 and 04/26/26, staff documented the bed alarm was not functioning properly. What was done for this? 7. On 04/12/26, [s/he] fell - was found on floor next to bed. Was the alarm sounding? 8. On 04/29/26, [s/he] fell, and the alarm did not sound - was this investigated as to why the alarm did not sound?" On 07/27/2026, Administrator A replied, via email, which read, in part: "6. On 3/29 there was a follow up note placed on 4/2 that the bed alarm was fixed. During that time the resident was placed on 30 minute checks to ensure safety. 7. After [his/her] alarm was found that the alarm was not working [Resident 1] was placed on 30 minute checks until [his/her] replacement alarm came in. We are unable to determine the cause of the alarm not working.	N 388		

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N 388	Continued From page 8 8. We investigated and found the alarm was not consistently working the alarm check was placed on hold and 30 minute checks were continued." Surveyor did not locate documentation to indicate Resident 1 was on 30-minute checks starting 03/29/2026 when staff first documented the alarm was not functioning properly. Resident 1's task administration record indicated staff began 30-minute checks on Resident 1 on 04/12/2026 at 6:30 p.m., after Resident 1 fell on 04/12/2026. The provider did not ensure Resident 1's ISP was implemented as written. Resident 1's bed alarm did not function properly.	N 388		
N 389	83.35(3)(d) Service plans updated annually or on changes Individual service plan review. Annually or when there is a change in a resident ' s needs, abilities or physical or mental condition, the individual service plan shall be reviewed and revised based on the assessment under sub. (1). All reviews of the individual service plan shall include input from the resident or legal representative, case manager, resident care staff, and other service providers as appropriate. The resident or resident ' s legal representative shall sign the individual service plan, acknowledging their involvement in, understanding of and agreement with the individual service plan. This Rule is not met as evidenced by: Based on record review and interview, the provider did not ensure Resident 1's individual service plan (ISP) was reviewed and revised	N 389		

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N 389	Continued From page 9 when there was a change in Resident 1's needs, abilities, physical and mental condition. Resident 1 had multiple falls, and his/her ISP was not reviewed and revised with additional fall interventions after each fall. Resident 1 was hospitalized and had required additional assistance with activities of daily living. Resident 1's ISP was not revised to reflect these changes until s/he was placed on hospice. Resident 1 had change in his/her mental health and sleeping pattern. Resident 1's ISP did not reflect these behaviors. Findings include: On 05/27/2026, the Department received a complaint with allegations that a resident was not receiving adequate care and services. On 07/23/2026 through 07/29/2026, Surveyor reviewed Resident 1's record. Resident 1's face sheet indicated s/he was admitted to the facility on 02/03/2026. His/her observation notes indicated s/he was admitted to the facility on 01/19/2026. S/he had multiple diagnoses including: brain aneurysm, cerebral vascular accident (stroke), mixed anxiety depressive disorder, and dementia with behavioral disturbance. Resident 1 had an activated health care power of attorney. FALLS On 07/27/2026, Surveyor reviewed the provider's fall policy and procedure titled, "FALL MANAGEMENT & FOLLOW-UP PROTOCOL." The policy and procedure read, in part:	N 389		

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N 389	Continued From page 10 "PURPOSE: While caring for the elderly, it is important to note the dementia population can be at heightened risk for falls. Commonly amongst individuals with dementia, a decline in gait or balance can be an indicator of decline in cognition. In middle and later stages, dementia can cause a decline in muscle strength, ambulation, and balance. Other contributing factors within the progression of dementia may include poor judgement (e.g., attempting to self-transfer when they require assistance), difficulty with communication (e.g., their need to utilize the bathroom, pain/discomfort, boredom, restlessness) and visual spatial problems (e.g., changes in floor color, misjudge steps). While some falls may be difficult to prevent, the goal of this policy is to ensure resident safety and support quality improvement efforts in reducing falls, and more specifically falls resulting in serious injury ...Fall Follow-Up Protocol ... 8. The resident Incident Report in [electronic health record] will be completed by the staff member who first responded to or witnessed (or RCC (Resident Care Coordinator) 3) the incident. Incident Investigation will be completed by RHC (Resident Health Coordinator) or supervisory-level Nursing Management ... Statements must be obtained by all staff who witnessed the incident. 9. Interventions are developed or revised based on the assessment and updated in the resident's Care Plan and Behavioral Evaluation, as applicable ...With All Falls - The Facility staff may implement the following steps in the incidence of a resident fall. 1) Find and identify the causing factor, implement immediate intervention related to cause, include details in the incident report of the cause, intervention, and effectiveness ... 2) If any changes in resident's baseline are noted, document in observation notes and notify	N 389		

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N 389	Continued From page 11 Manager on-call. Manager on-call will assist in triage to determine appropriate plan of action. Consult with DON (Director of Nursing) (if applicable). 3) Update Care Plan. 4) Notify Family/POA (power of attorney)/MCO (managed care organization)/PCP (primary care provider) ... include causing factor, immediate intervention, education, and recommendations ..." Resident 1's ISP, titled "30 Day Assessment," dated 02/26/2026, signed by RHC D, read, in part: " Resident utilizes a 4 wheeled walker for ambulation ... Resident is independent with ambulation throughout the facility ... Resident is currently independent with transitioning between lying down, sitting, and standing positions ... Resident is at risk for falls based upon Fall Risk Evaluation due to multiple factors including, diagnosis of Dementia [sic], medications and they [sic] had 2 falls last winter. Resident's Desired Goals & Outcomes: To remain free from serious injury related to falls. Service Provider Responsibilities: Staff to monitor for any changes that may increase risk for falls ... Resident has the following interventions in place to reduce risk of serious injury from falls: gripper socks, night light in room. Resident's Desired Goals & Outcomes: To remain free from serious injury from falls. Service Provider Responsibilities: Staff to ensure interventions are put in place and followed to reduce risk from serious injury related to falls." Surveyor reviewed Resident 1's incident reports, which included the following falls: 03/02/2026 at 10:04 a.m. - "Staff were returning to quad after water pass and found resident laying on the floor in front of [his/her] bathroom door, laying on left side attempting to grab at the	N 389		

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N 389	Continued From page 12 door/wall ... When asked what happened resident stated 'I don't know, my legs hurt' Nurse did range of motion and visibly inspected residents [sic] legs, no concerns on skin. 03/11/2026 at 8:48 a.m. - "writer entered room to wake resident for breakfast. writer [sic] observed resident laying on floor between bathroom and bedroom. resident [sic] was assessed for pain and stated 'ouch' when touching right hip, manager was called and directed staff to call EMS (emergency medical services) ... resident repeatedly said 'ouch' while touching right hip. resident was not able to state what had occurred." On 03/11/2026 at 3:00 p.m., Registered Nurse (RN) C documented: "Resident arrived back from hospital via family transport at approx.. (approximately) 11:50am. No new orders. Findings from fall no injury sustained. Labs remarkable, cervical and head CT (computed tomography) Negative. Vitals stable and WNL (within normal limits) for this resident." On 03/13/2026 at 9:17 a.m. - "Witnessed resident walk to bathroom without walker, writer followed with walker and saw resident lose balance as resident was shutting the door to the bathroom. Resident fell on left hip and cut [his/her] finger with own nail to point of breaking skin. Writer held resident up right sitting until other caregivers responded to call. Resident 1's ISP, titled "Other: Bed alarm," dated 03/13/2026, signed by RHC D, read, in part: "Resident utilizes a 4 wheeled walker for ambulation ... Resident is independent with ambulation throughout the facility ... Resident is currently independent with transitioning between	N 389		

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(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETE DATE
N 389	Continued From page 13 lying down, sitting, and standing positions ... Resident is at risk for falls based upon Fall Risk Evaluation due to multiple factors including, diagnosis of Dementia [sic], medications and they [sic] had 2 falls last winter. Resident's Desired Goals & Outcomes: To remain free from serious injury related to falls. Service Provider Responsibilities: Staff to monitor for any changes that may increase risk for falls ... Resident has the following interventions in place to reduce risk of serious injury from falls: gripper socks, night light in room. Resident's Desired Goals & Outcomes: To remain free from serious injury from falls. Service Provider Responsibilities: Staff to ensure interventions are put in place and followed to reduce risk from serious injury related to falls ... Resident utilizes a bed alarm. Resident needs this to alert staff when resident is attempting to transfer without assistance and to minimize the risk of serious injuries related to falls. Resident's Desired Goals & Outcomes: Resident desires to remain free from serious injuries or incidents from occurring. Service Provider Responsibilities: Staff" (Staff was the only word after Service Provider Responsibilities). The ISP did not reflect that Resident 1 had 3 falls since his/her 30-day ISP was developed. The ISP indicated s/he could transfer independently, but the alarm was used to alert staff when resident attempted to transfer independently. From 03/17/2026 until 03/23/2026, Resident 1 was hospitalized. On 03/24/2026, RN C documented: "Resident assessed and vitals taken per [family] request. WNL for this resident. Resident is now a two person transfer as [s/he] is still deconditioned from hospital stay ..."	N 389		

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N 389	Continued From page 14 Resident 1's ISP, titled "Other: Transfers," dated 03/24/2026, signed by RHC D, read, in part: "Resident utilizes a 4 wheeled walker for ambulation ... Mobility -Stand by assistance with 2 staff ...Resident needs 2 staff members and stand by assistance to transition between lying down, sitting, and standing positions ..." On 04/12/2026 at 2:21 p.m. - "Resident was yelling, staff went in and found [him/her] on the floor next to [his/her] bed." The incident report did not indicate if the alarm was sounding. On 04/12/2026, 30-minute safety checks were documented on Resident 1's task administration record starting at 6:30 p.m. On 04/29/2026 at 2:31 a.m. - "As writer and staff were entering hallway, writer and staff heard resident yelling for help. Resident was wedged between their bed side table and their bed sitting in an upright position with their body faced towards the wall and their back towards their room. Resident was sitting on their legs and feet. Writer and staff asked what happened, resident was unable to provide clear response. After performing range of motion, resident was transferred to their bed and their vitals were collected. No visible marks or scratches were present, doesn't has [sic] no visible indication of hitting their head but ultimately unclear response when asked. When transferring, resident began to dry heave and had an [sic] very small amount of emesis. Resident winced, clenched teeth, and said 'ow' a few times but once vitals were gathered and they [sic] were covered in bed, they [sic] appeared far less uncomfortable. On-Call contacted as well as [family] (POA) ... Previous shift stated supervisors were aware of lack of bed	N 389		

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N 389	Continued From page 15 alarm for resident. When contacted, supervisor informed writer they had not been made aware and instructed writer to monitor more frequently, that they'd have a look at things in the morning." Resident 1's ISP was not reviewed and revised again until 05/07/2026. Resident 1's ISP, titled "Change in Condition: Hospice Admission," dated 05/07/2026, signed by RHC D, indicated Resident 1 was now dependent on staff for his/her activities of daily living. Resident 1 used a wheelchair for mobility and was dependent on staff for mobility. The ISP read, in part: "Resident is at risk for falls based upon Fall Risk Evaluation due to multiple factors including, diagnosis of Dementia [sic], medications and they had 2 falls last winter ... Resident has the following interventions in place to reduce risk of serious injury from falls: gripper socks, night light in room, crash mat, pad alarm ... Resident utilizes a bed alarm. Resident needs this to alert staff when resident is attempting to transfer without assistance and to minimize the risk of serious injuries related to falls. Resident's Desired Goals & Outcomes: Resident desires to remain free from serious injuries or incidents from occurring. Service Provider Responsibilities: Staff" On 05/10/2026, Resident 1 died. On 07/27/2026, Surveyor emailed Administrator A with questions related to Resident 1's falls. Surveyor asked if Resident 1's ISP was updated after each of Resident 1's falls. On 07/27/2026, Administrator A replied, via email, in part: "We don't update the care plan for every fall, [sic] We have a fall interventions section for residents that are at risk for falls. [S/he] has the following	N 389		

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N 389	Continued From page 16 intervention (Enter here) and the next time it was updated was on 3/13 a bed alarm was placed." The provider did not ensure Resident 1's ISP was reviewed after each fall to revise or develop new fall interventions to prevent future falls. ACTIVITIES OF DAILY LIVING (ADL's) Resident 1's ISP, titled "30 Day Assessment," dated 02/26/2026, signed by RHC D, read, in part: "Resident prefers a shower and can get in and out independently. Resident needs cues and sometimes hands on assistance with completing all bathing tasks. Resident prefers to shower late at night. Staff to assist with applying lotion to skin following bathing ... Resident is independent with picking out clothing and with dressing in the morning and before bed. Resident's clothing is sometimes inside out or backwards. May need assisting reversing their clothing on occasion ... Resident is independent with all grooming tasks. Resident can wash face, wash up for morning and bedtime cares, apply deodorant, and comb hair independently ...Resident can trim, file, and clean nails independently ... Resident can complete all oral cares independently ... Resident is on a 4-hour toileting program due to incontinence of urine. Resident needs assistance with changing incontinent products when soiled and staff assistance with peri cares after each incontinent episode ... Resident utilizes a 4 wheeled walker for ambulation ... Resident is independent with ambulation throughout the facility ...Resident is currently independent with transitioning between lying down, sitting, and standing positions ... Resident is able to eat independently" From 03/17/2026 until 03/23/2026, Resident 1 was hospitalized.	N 389		

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N 389	Continued From page 17 On 03/24/2026, RN C documented: "Resident assessed and vitals taken per [family] request. WNL for this resident. Resident is now a two person transfer as [s/he] is still deconditioned from hospital stay ..." Resident 1's ISP, titled "Other: Transfers," dated 03/24/2026, signed by RHC D, read, in part: "Resident utilizes a 4 wheeled walker for ambulation ... Mobility -Stand by assistance with 2 staff ...Resident needs 2 staff members and stand by assistance to transition between lying down, sitting, and standing positions ..." There were no other revisions made in the ISP related to Resident 1's ADLs. On 04/05/2026, staff documented: "Resident was supposed to get a shower today. Writer and lead decided it wasn't a great idea to do the shower tonight because resident was very sleepy and couldn't keep [his/her] eyes open or [his/her] body straight up. Resident kept falling over to the side and we didn't want [him/her] to fall in the shower." On 04/10/2026, staff documented: "Rc'd (Received) new order from [name] Please place resident on mechanical soft diet due to having difficulty chewing food." Resident 1's ISP was revised on 04/10/2026 to indicate s/he was on a mechanical soft diet. The ISP, dated 04/10/2026, read, in part: "Resident has a mechanical soft diet due to chewing difficulties. Food should be soft, moist, bite sized pieces ...Resident is able to eat independently." On 04/10/2026 at 8:45 p.m., staff documented, in part: " ... Resident was leaning forward the majority time in [his/her] wheelchair. Staff	N 389		

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N 389	Continued From page 18 reminded resident to sit back multiple times due to [his/her] safety. Resident listened to staff for a minute then went back to leaning heavily forward." On 04/11/2026 at 2:30 p.m., staff documented: " ... Resident is heavily leaning forward and refuses to sit back in wheelchair, so [s/he] does not fall forward ..." On 04/15/2026 at 9:45 a.m., staff documented: "Resident has had continual struggles to fed [sic] [him/herself] during mealtime, resident will say [s/he] would like to eat but can only manage a spoonful or two until attempting to sleep. RCC assisted in feeding until [s/he] declined food ..." On 04/18/2026 at 1:30 p.m. - "For lunchtime staff were able to find enough spare pillows to prop resident upright ... Resident only consumed the liquid from [his/her] chicken noddle soup, as anything solid put into [his/her] mouth [s/he] would pocket into [his/her] cheek without fully chewing. Resident was able to swallow small sips of water when offered on a sponge swab ..." 04/19/2026 at 7:45 a.m., staff documented, in part: "Resident was transferred to their wheelchair in hopes that change of position would relax them ..." On 05/01/2026 at 12:00 a.m., staff documented, in part: " ...resident was given a shower ... Resident was washed up thoroughly and had [his/her] hair washed as well. Resident was then dressed into pajamas ..." On 05/03/2026 at 6:30 p.m., staff documented, in part: " ... Resident kept falling asleep during the feeding process ..."	N 389		

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N 389	Continued From page 19 On 05/06/2026 at 1:30 a.m., staff documented, in part: " ... Resident was checked for incontinence and blankets were adjusted. Resident was also re-positioned." On 05/07/2026, Resident 1's ISP was reviewed and revised for change of condition. The ISP indicated Resident 1 was dependent on staff for ADLs and utilized a wheelchair. Prior to 05/07/2026, Resident 1's ISP indicated s/he was independent with his/her ADL's. On 05/10/2026, Resident 1 died. On 07/27/2026 at approximately 3:30 p.m., Surveyor interviewed Person B on the phone. Person B was Resident 1's activated health care power of attorney. Person B told Surveyor Resident 1 never received nail care. S/he said they never clipped or cleaned Resident 1's nails underneath them. Person B told Surveyor after Resident 1 was hospitalized, s/he was unable to feed him/herself. S/he said some of the staff would bring Resident 1 to the table, set his/her tray in front of Resident 1 and walk away. Person B told Surveyor Resident 1's hair was never brushed, and his/her teeth were never brushed. Person B said, "That was not my [mother/father]." Person B said they just left Resident 1 in his/her room, they did not do cares for him/her. S/he said the more Resident 1's cares were, the less staff did for him/her. Surveyor asked if Resident 1 was ambulatory after s/he was hospitalized. Person B said Resident 1 did not walk after s/he was hospitalized. Person B said Resident 1 was in a wheelchair and needed assistance with everything. Person B said Resident 1 could shuffle his/her feet while in the wheelchair to get around.	N 389		

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N 389	Continued From page 20 Resident 1's ISP did not reflect his/her need with assistance with ADLs after s/he was hospitalized until 05/07/2026, 3 days before s/he died. Resident 1's legal representative told Surveyor staff did not assist with ADLs, including grooming, eating, and brushing his/her teeth. MENTAL HEALTH - BEHAVIORS Resident 1's ISP, dated from 02/26/2026 through his/her ISP dated 05/07/2026, indicated staff administered all medications. The ISPs read, in part: "Resident takes Pregabalin, Escitalopram, Haloperidol, and Lorazepam. All psychotropic medications have medication consents signed by my POA (power of attorney) and are monitored quarterly by the pharmacy and nurse." Under the section titled, "Psychological Wellbeing" No behaviors were marked. This section read, in part: "Resident sleeps through the night ...Resident requires the following interventions and redirection strategies when behaviors arise smile, be gentle, talk to [him/her] about the corner store [s/he] use to own, suggest doing puzzles, talking about dogs/cats, color, talking about gardening and baking." On 04/03/2026, Registered Nurse (RN) C documented: "Received a call from PCP (primary care provider) office stating they would be sending over an order to discontinue scheduled Haldol per POA - [son/daughter] request. New order written for Haldol 1mg (milligram) tablet PO (by mouth) PRN (as needed) at HS (hour of sleep) for agitation ..." On 04/13/2026 at 12:30 a.m., staff documented Resident 1 was attempting to walk into another resident's room. At 1:15 a.m., staff documented:	N 389		

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N 389	Continued From page 21 "Resident even more restless than previously charted. Every time RCC (Resident Care Coordinator) repositions resident back in [his/her] bed with both of [his/her] feet in the bed. Resident will start to scream to someone (a family member perhaps) to 'come back inside' after RCC leaves from repositioning. Resident keeps putting themselves half in and out of the bed within moments of RCC leaving. This has gone on since the beginning of shift ..." On 04/15/2026 at 2:30 a.m., staff documented: "Resident got out of bed while RCC was preoccupied with another resident. Resident was trying to get UNDER [his/her] bed to stop the noise and look for something ..." On 04/16/2026 at 5:45 a.m., staff documented: "Resident throughout the night was restless (screaming, etc)" On 04/18/2026 at 10:15 p.m., staff documented: "Resident was very agitated and restless tonight. [S/he] said things were crawling on [him/her] and had a thought of carving pumpkins. PRN haloperidol was given." 04/18/2026 at 11:15 p.m., staff documented: "Resident continues to get out of bed and appears to be restless. Resident is also very agitated, and [his/her] PRN Haloperidol was ineffective. On call was notified of this ..." 04/19/2026 at 1:30 a.m., staff documented: "Resident has been very vocal tonight and has not slept so far. Resident has been repetitively shouting incoherent rambles. Resident talked about a toaster, someone licking [him/her], [his/her] arm and teeth hurting as well as [his/her] legs and back being itchy. Resident was given	N 389		

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N 389	Continued From page 22 Haloperidol at shift change for agitation, which was ineffective ..." On 04/22/2026 at 6:00 a.m., staff documented: "Resident was very vocal and restless again tonight up until 1am ..." On 04/22/2026 at 11:45 p.m., staff documented: "Resident has had many attempts and refusals of laying down. Resident is insistent on staying up right. Has conversations, loudly, with [him/herself] that are hard to understand and that are disturbing to [his/her] immediate neighbor due to them trying to sleep. RCC gave a PRN Haloperidol after resident showed agitation when it came to multiple reattempts to lay resident back down safely in [his/her] bed. Resident has expressed a desire for alcohol and 'pop' during these conversations and wants 'Daddy.' Has conversations with [him/herself] there in about where [s/he] is and etc. When any RCC approaches resident seems to think [s/he] is in a bar and is requesting alcohol. Becomes agitated when RCC gives water ..." 04/24/2026 at 9:45 p.m., staff documented: "Resident was given a prn haloperidol due to resident grabbing writers [sic] arm and digging [his/her] nails into writer's arm ..." On 04/26/26 at 2:15 a.m., staff documented, in part: "Resident has been yelling and talking to [him/herself] quite loudly since shift start. PRN was administered and it does not seem to be effective." On 05/03/2026 at 2:45 a.m., staff documented: "Resident has been vocal on and off tonight. Resident has slept about 2 hours in total. Resident was heard talking incoherently ... Writer	N 389		

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N 389	Continued From page 23 administered PRN Haloperidol for agitation, but it does not appear to have given any relief ..." 05/03/2026 at 4:30 a.m., staff documented: "Resident has not slept since previous observation and continues to speak loudly and incoherently ..." On 05/04/2026 at 11:45 p.m., staff documented, in part: "Writer attempted to administer a PRN Haloperidol due to their increased volume and ramblings becoming angrier, however resident spat it out on numerous occasions ..." On 05/06/2026 at 1:30 a.m., staff documented: "Resident has been vocal since arrival to shift. Resident keeps talking about [his/her] mommy and daddy and other incoherent conversations ..." On 05/10/2026, Resident 1 died. Resident 1's ISP did not accurately reflect his/her sleeping pattern as staff documented Resident 1 did not sleep through the night. His/her ISP did not reflect behaviors that would indicate the use for an as needed psychotropic medication, haloperidol (Haldol). Cross Reference: N0408 DHS 83.37(1)(i) PRN Psychotropic Medication	N 389		
N 408	83.37(1)(i) PRN psychotropic medication. As needed (PRN) psychotropic medication. When a psychotropic medication is prescribed on an as needed basis for a resident, the CBRF shall do all of the following: 1. The resident ' s individual service plan shall include the rationale for use and a detailed description of the	N 408		

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N 408	Continued From page 24 behaviors which indicate the need for administration of PRN psychotropic medication. 2. The administrator or qualified designee shall monitor at least monthly for the inappropriate use of PRN psychotropic medication, including but not limited to, use contrary to the individual service plan, presence of significant adverse side effects, use for discipline or staff convenience, or contrary to the intended use. 3. Documentation in the resident ' s record shall include the rationale for use, description of behaviors requiring the PRN psychotropic medication, the effectiveness of the medication, the presence of any side effects, and monitoring for inappropriate use for each PRN psychotropic medication given. This Rule is not met as evidenced by: Based on record review and interview, the provider did not ensure Resident 1's individual service plan (ISP), who was prescribed a PRN (as needed) psychotropic medication, included the rationale for use and a detailed description of the behaviors which indicate the need for administration of PRN psychotropic medication. Findings include: On 05/27/2026, the Department received a complaint with allegations that a resident was not receiving adequate care and services, including medication administration. On 07/23/2026, Surveyor reviewed Resident 1's record. Resident 1 was admitted to the facility on 02/03/2026. Resident 1 had multiple diagnoses including: brain aneurysm, cerebral vascular accident (stroke), mixed anxiety depressive	N 408		

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N 408	Continued From page 25 disorder, and dementia with behavioral disturbance. Resident 1's ISP, dated 04/10/2026 and 05/07/2026, indicated staff administered all medications. The ISPs read, in part: "Resident takes Pregabalin, Escitalopram, Haloperidol, and Lorazepam. All psychotropic medications have medication consents signed by my POA (power of attorney) and are monitored quarterly by the pharmacy and nurse." Under the section titled, "Psychological Wellbeing" No behaviors were marked. This section read, in part: "Resident sleeps through the night ...Resident requires the following interventions and redirection strategies when behaviors arise smile, be gentle, talk to [him/her] about the corner store [s/he] use to own, suggest doing puzzles, talking about dogs/cats, color, talking about gardening and baking." On 04/03/2026, Registered Nurse (RN) C documented: "Received a call from PCP (primary care provider) office stating they would be sending over an order to discontinue scheduled Haldol per POA - [son/daughter] request. New order written for Haldol 1mg (milligram) tablet PO (by mouth) PRN at HS (hour of sleep) for agitation ..." Resident 1's medication administration record (MAR) for April 2026 indicated Resident 1 received 4 doses of the PRN haloperidol. Resident 1's MAR for May 2026 indicated s/he received 3 doses of PRN haloperidol. On 07/27/2026, Surveyor emailed Administrator A, which included questions related to Resident 1. Surveyor asked if Resident 1's ISP was updated when his/her Haldol was changed from scheduled to PRN to include the rationale for use and a	N 408		

Wisconsin Department of Health Services

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: 0020912	(X2) MULTIPLE CONSTRUCTION A. BUILDING: _____ B. WING: _____		(X3) DATE SURVEY COMPLETED C 07/29/2026
NAME OF PROVIDER OR SUPPLIER WESTON MEMORY CARE			STREET ADDRESS, CITY, STATE, ZIP CODE 3404 COMMUNITY CENTER DRIVE WESTON, WI 54476		
(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETE DATE	
N 408	Continued From page 26 detailed description of the behaviors for when staff should use the PRN. On 07/27/2026, Administrator A replied, via email, which read, in part: "The care plan was not updated to reflect the PRN change at that time." The provider did not ensure Resident 1's ISP was updated when s/he was prescribed a PRN psychotropic medication, haloperidol (Haldol), to include a rationale for use and a detailed description of Resident 1's behaviors which would indicate the need for administration of PRN haloperidol.	N 408			